NCCP 00212 · VERSION 7

Your treatment, explained

Metastatic colorectal cancer · bevacizumab component with fluoropyrimidine-based chemotherapy · NCCP 00212 v7 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Bevacizumab 10 · colorectal

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment stages
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Why bevacizumab is included
- Before treatment starts
- During and between cycles
- Living with treatment
- Review and follow-up
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

Three medicines. Different jobs.

To help control bowel cancer that has spread, alongside the chemotherapy chosen by your oncology team.

ANTI-VEGF ANTIBODY

Bevacizumab

Bevacizumab



Blocks the VEGF growth signal

Bevacizumab binds VEGF, a signal that helps new blood vessels grow. It can slow tumour growth but also affect normal blood vessels and wound healing.

Blocks the VEGF growth signal → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Report bleeding, severe tummy pain, unusual headache or vision changes urgently. Tell the team about any surgery or wound.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Head / blood pressure

Bevacizumab

New severe headache, confusion or visual change needs urgent advice.

3 Lung bleeding

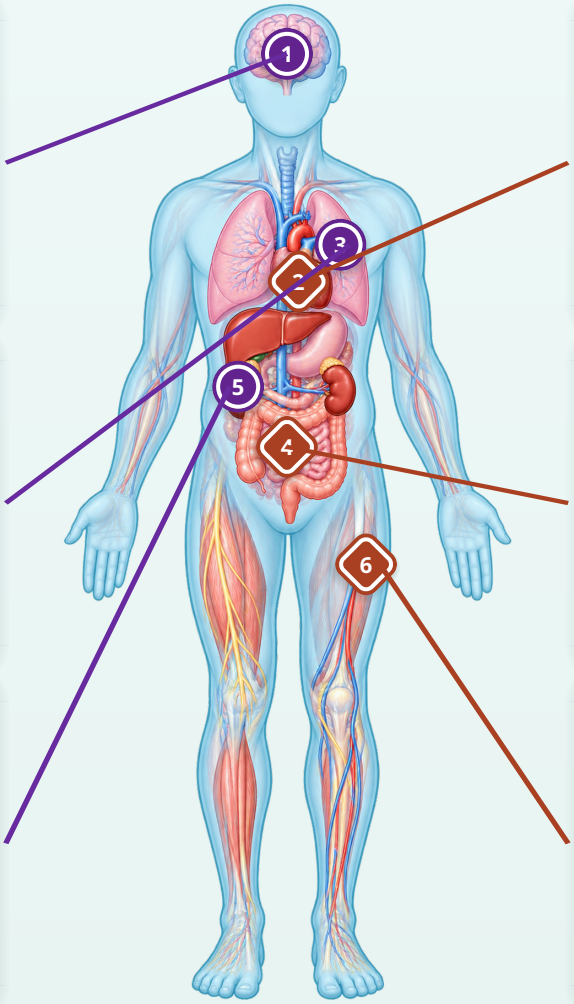
Bevacizumab

Coughing up blood needs immediate assessment.

5 Urine / kidneys

Bevacizumab

Urine tests detect protein; report swelling or much less urine.



2 Heart / blood vessels

Bevacizumab

Chest pain, sudden breathlessness or a swollen painful leg needs urgent help.

4 Bowel / abdomen

Bevacizumab

Severe tummy pain or a rigid swollen abdomen needs urgent assessment.

6 Bleeding / wounds

Bevacizumab

Report bleeding and unhealed wounds; tell the team before procedures.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Treatment → review → next steps

14-DAY REFERENCE CYCLE

Bevacizumab with chemotherapy

Review

Bevacizumab is given by drip on Day 1 every 14 days with the prescribed fluoropyrimidine-based chemotherapy. Treatment continues while helping and tolerable; there is no fixed total number of cycles. The partner chemotherapy has its own dose days, tests and support plan.

Your team confirms each stage and discusses any changes with you.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

Check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment	Confirm colorectal indication / partner	Bevacizumab Day 1 every 14 days	Follow partner prescription	Review benefit and tolerability	Next plan agreed individually
Bloods / urine	FBC, kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart	BP and cardiac history/ exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests	Individual disease plan	No fixed scan interval	INR if warfarin	Response review	Follow-up agreed

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00212 v7 · official regimen · Avastin current EMA product information

THE WHOLE TREATMENT JOURNEY

Bevacizumab 10 · colorectal: treatment and checks

Metastatic colorectal cancer · bevacizumab component with fluoropyrimidine-based chemotherapy

Bevacizumab is given by drip on Day 1 every 14 days with the prescribed fluoropyrimidine-based chemotherapy. Treatment continues while helping and tolerable; there is no fixed total number of cycles. The partner chemotherapy has its own dose days, tests and support plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00212	Confirm colorectal indication / partner	Bevacizumab Day 1 every 14 days	Follow partner prescription	Review benefit and tolerability	Next plan agreed individually
Bloods / urine NCCP 00212	FBC, kidney/liver, urine protein	Regular blood/urine checks	Quantify protein if needed	Review trends	Follow abnormalities
Blood pressure / heart NCCP 00212	BP and cardiac history/ exam	BP before each cycle	Extra BP if raised	Review control	BP post-treatment
Scans / other tests NCCP 00212	Individual disease plan	No fixed scan interval	INR if warfarin	Response review	Follow-up agreed

What each check is for

1

Bevacizumab with chemotherapy

14-day reference cycle

To help control bowel cancer that has spread, alongside the chemotherapy chosen by your oncology team.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00212

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.34

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Bevacizumab is given by drip on Day 1 every 14 days with the prescribed fluoropyrimidine-based chemotherapy. Treatment continues while helping and tolerable; there is no fixed total number of cycles. The partner chemotherapy has its own dose days, tests and support plan.

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Bevacizumab infusion Day 1 every 14 days after review	Day 1													
Separate chemotherapy schedule	Use the named partner pack for tablet days, infusions or pump timing													
Symptoms and safety checks	Symptoms can occur at any time; attend arranged tests													

Partner treatment changes the schedule: This calendar shows bevacizumab only. Fluoropyrimidine-based chemotherapy may involve tablets, other infusion days or a home pump. Use its separate written calendar.

Treatment decisions: The team may hold or stop bevacizumab for side effects. Decisions for each chemotherapy medicine are separate; do not restart or alter treatment yourself.

Symptoms can occur at any time: Raised blood pressure or protein in urine may have no symptoms. Attend tests, and report bleeding, severe abdominal pain, chest pain, breathlessness or new neurological symptoms promptly.

> **Why these bands stay broad:** Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Read the whole cycle as text

Day 1: Infusion and checks

Bevacizumab after review; the chemotherapy partner follows its own prescription. Confirm blood pressure, urine testing and the full combination plan.

Day 2–14: Between bevacizumab visits

Follow the separate chemotherapy calendar and report new symptoms promptly. Keep arranged checks; contact oncology if unwell.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.



STEP 2

Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.



STEP 3

Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.



STEP 4

Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00212 v7 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Partner chemotherapy

This covers bevacizumab only. Bring the separate chemotherapy schedule, anti-sickness plan and urgent-contact instructions.

2 Blood pressure and urine

These checks can find problems before you feel unwell. Take blood-pressure medicines as prescribed; do not adjust them yourself.

3 Surgery and dental procedures

Tell oncology before any operation or invasive procedure. The team must agree treatment pauses and assess wound healing.

4 Support medicines

Bevacizumab alone does not usually need routine anti-sickness prevention. Chemotherapy partners may need a different plan.

Sources: nccp · ncis · antiemetic

TREATMENT REFERENCE · 01

Why bevacizumab is included

This is one part of your combination treatment.

Treatment aim

To help control bowel cancer that has spread, alongside the chemotherapy chosen by your oncology team.

Sources: nccp

How it works

Bevacizumab binds VEGF, a signal that helps new blood vessels grow. It can slow tumour growth but also affect normal blood vessels and wound healing.

Sources: nccp

Which evidence applies?

E3200 studied bevacizumab 10 mg/kg every two weeks with FOLFOX4 in metastatic colorectal cancer previously treated with a fluoropyrimidine and irinotecan. Adding bevacizumab improved survival in that trial. This does not establish the same result with every chemotherapy partner or predict an individual outcome.

Sources: trial

TREATMENT REFERENCE · 02

Before treatment starts

Some problems are found by tests before symptoms develop.

Baseline checks

Blood count, kidney and liver tests, urine protein, blood pressure and a heart history/examination are checked. A heart ultrasound is considered after chest-wall radiotherapy or previous anthracycline treatment.

Sources: nccp

Medicines, wounds and procedures

Bring all medicines and supplements. Report bleeding, clotting problems, poorly controlled blood pressure, unhealed wounds and planned operations or dental procedures. Oncology must coordinate any treatment pause.

Sources: nccp

The complete combination

Confirm the chemotherapy partner, its tests, infusion or tablet calendar, any pump arrangements and prescribed anti-sickness plan. This bevacizumab pack cannot replace those instructions.

Sources: nccp

TREATMENT REFERENCE · 03

During and between cycles

Your next dose depends on review, not just the date.

Blood pressure and urine

Blood pressure is checked before each cycle and after treatment. Regular blood-count, kidney/liver and urine-protein tests continue. Extra tests are arranged for abnormal results.

Sources: nccp

Warfarin

If taking warfarin, INR is checked weekly until the warfarin dose is stable, then before each cycle. Tell the team about bleeding or any medicine changes.

Sources: nccp

Symptoms that need urgent help

Call your oncology team immediately for coughing blood, significant bleeding, severe tummy pain, sudden breathlessness or chest pain, or a new severe headache, confusion, seizure or vision change. Fever or sudden illness also needs urgent advice, especially with chemotherapy.

Sources: nccp

Support medicines

Bevacizumab alone has minimal sickness risk. The chemotherapy partner determines additional anti-sickness treatment and other supportive medicines; use the issued prescription.

Sources: nccp

TREATMENT REFERENCE · 04

Living with treatment

Keep information and contacts available on paper as well as online.

Before any procedure

Tell every treating clinician that you receive bevacizumab. It can affect wound healing; your oncology team must agree timing around operations and check healing before restarting.

Sources: nccp

Keeping a record

This is a rolling record, not a six-cycle limit. Record actual doses and interruptions. A stamp records completed treatment and does not authorise the next dose.

Sources: nccp

Pregnancy and fertility

Discuss fertility and pregnancy prevention with your team before treatment. Check the duration of contraception and breastfeeding restrictions for every medicine in the combination; contact oncology promptly if pregnancy is possible.

Sources: nccp

TREATMENT REFERENCE · 05

Review and follow-up

There is no universal final cycle or scan timetable.

Assessing benefit

Your consultant sets scans and any tumour-marker tests according to your cancer, symptoms and treatment plan. These are separate from routine blood-pressure and laboratory safety checks.

Sources: nccp

After a pause or stopping

Continue arranged blood-pressure checks and follow-up of unresolved effects. Ask who reviews results, what treatment follows and whom to contact if symptoms develop.

Sources: nccp

Evidence in this colorectal setting

The chemotherapy partner, dose schedule and previous treatment determine how trial results apply.

E3200 studied bevacizumab 10 mg/kg every two weeks with FOLFOX4 in metastatic colorectal cancer previously treated with a fluoropyrimidine and irinotecan. Adding bevacizumab improved survival in that trial. This does not establish the same result with every chemotherapy partner or predict an individual outcome.

Sources: trial

PREPARE FOR THE CONVERSATION

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00212 v7. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00212 v7 · official regimen
- Avastin current EMA product information
- NCCP antiemetic classification v 6 and oral pathway
- NCIS medical oncology antiemetic medicines
- E3200: bevacizumab with FOLFOX4 after previous treatment

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6552/212_Bevacizumab_10.pdf

https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf

https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/17442997/